

HISTORY & PHYSICAL — Internal Medicine

Patient: Jamil A. Robinson DOB: 09/30/1971 MRN: IM-554210

Insurance: Anthem BCBS / W-77231906 / Group 88410

Provider: Dr. Sasha Petrova, MD NPI: 1665789023

Chief complaint:

54 y/o M with progressive dyspnea on exertion x 3 months, now occurring at rest. Associated with bilateral lower extremity swelling.

History of present illness:

Symptoms began gradually in January. Patient initially attributed to deconditioning. Over the past 4 weeks, dyspnea has worsened and now occurs after walking <100 feet. Reports orthopnea, requires 3 pillows to sleep. Two episodes of PND in past week. Lower extremity swelling worse in evenings, improves with elevation. Denies chest pain, syncope, palpitations, hemoptysis.

Past medical history:

HTN (15 years), T2DM (8 years), HLD, OSA on CPAP, prior MI 2018 s/p PCI to LAD.

Medications: lisinopril 40 mg, metoprolol 50 mg BID, atorvastatin 80 mg, metformin 1000 mg BID, aspirin 81 mg, ezetimibe 10 mg.

Physical exam:

Vitals: BP 158/94, HR 98, RR 22, SpO2 92% RA, wt 102 kg.

Gen: mild respiratory distress at rest.

Neck: JVD to 10 cm. CV: tachy, S3 gallop, no m/r/g.

Lungs: bibasilar crackles to mid-lung fields.

Ext: 2+ pitting edema to mid-shin bilaterally.

Labs / Studies:

BNP 1820, troponin <0.01, Cr 1.4 (baseline 1.0), Na 134, K+ 4.6.

CXR: cardiomegaly, bilateral pulmonary edema.

EKG: NSR, LBBB (new vs. 2018), no acute ischemic changes.

Echo: EF 25% (was 45% in 2019), severe global LV hypokinesis.

Assessment:

1. Acute decompensated heart failure with reduced EF (I50.21).
2. Hypertensive heart disease (I11.0).
3. Type 2 diabetes mellitus, uncontrolled (E11.65).
4. Hyperlipidemia (E78.5).
5. Obstructive sleep apnea on CPAP (G47.33).
6. Acute kidney injury, likely cardiorenal (N17.9).

Plan:

- Admit to cardiology for IV diuresis (furosemide 80 mg IV BID).
- Initiate sacubitril/valsartan, target dose over 2 weeks.
- Continue metoprolol, hold lisinopril given AKI.
- Daily weights, strict I/O, low-sodium diet (<2g/day).
- Echo with strain in 1 week.
- Cardiology f/u in 2 weeks post-discharge.
- Patient/family education on HF self-management.